

1 **Apparent Catamenial Epilepsy Classification Under a Null Simulation**

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17 data, analyses, interpretation, and conduct of the research; had full access to all output data; and
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20 visualization, writing-original draft, writing-review and editing, funding acquisition; SC,
21 methodology, clinical interpretation, writing-review and editing; WK, methodology, clinical
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23 review and editing.

24 **Abstract**

25 Objective: To quantify apparent catamenial epilepsy classification under a null model in which
26 seizure and menstrual-cycle diaries are independent, with emphasis on C1, C2, and C3 pattern-
27 specific false positives. Methods: We simulated 100,000 synthetic participants for 36 months in
28 healthy ovulatory and heterogeneous menstruating-age cohorts. CHOCOLATES generated
29 seizure diaries and HORMONE-CYCLE generated independent menstrual/hormone diaries. A
30 random circular shift was applied before merging. Strict Herzog phase labels were primary, with
31 a luteal-anchored fixed periovulatory sensitivity. We evaluated exact Herzog 2004, windowed
32 Herzog thresholds, Herzog plus ≥ 4 seizure-day minimums, and cycle reproducibility $\geq 2/3$
33 across calendar, cycle, and full-diary windows; negative-binomial C1/C2 regression was
34 evaluated as a full-diary calibrated comparator. Results: In 36-month strict-Herzog windows,
35 windowed Herzog thresholds classified 11.2% of healthy ovulatory and 36.3% of heterogeneous-
36 cohort participants as apparent catamenial epilepsy under independence. In the heterogeneous
37 cohort, C3 drove the excess: the C1/C2 union was 11.8%, whereas C3 positivity occurred in
38 37.0% of applicable windows. Negative-binomial C1/C2 regression exhibited approximately
39 nominal Type I error, 4.1% and 4.2%. Three-month calendar windows remained vulnerable, with
40 windowed Herzog false-positive rates of 41.3% and 51.2%. Conclusions: Apparent catamenial
41 epilepsy classification can be common under independence, especially when C3/inadequate-
42 luteal-phase logic is applied in heterogeneous cycles. Studies should report C1, C2, and C3
43 separately and prespecify diary duration, seizure-day minimums, denominator handling, phase
44 labeling, and reproducibility criteria.

45 Classification of Evidence: Not applicable. This simulation study evaluates operating
46 characteristics of classification rules under a synthetic null model and does not test a diagnostic
47 intervention or therapeutic effect in human participants.

48 Search Terms: catamenial epilepsy; epilepsy; seizures; menstrual cycle; simulation study.

49

50 **Introduction**

51 Catamenial epilepsy describes seizure exacerbation in relation to the menstrual cycle. The
52 clinical literature has long recognized perimenstrual, periovulatory, and inadequate luteal-phase
53 patterns, but reported prevalence varies widely because studies use different phase definitions,
54 thresholds, diary durations, and eligibility rules. Herzog and colleagues reported 39.1%
55 catamenial epilepsy in women who charted seizures and menses for three complete cycles using
56 a two-of-three-cycle rule, and later work from the NIH Progesterone Treatment Trial has been
57 interpreted as supporting prevalence near 44% in selected trial participants.^{1,5}
58 The diagnostic problem is not only biological. Seizures are temporally structured: clustering,
59 circadian rhythms, and multidien rhythms have been documented in long-term diaries and
60 intracranial recordings.⁹⁻¹¹ If seizure rhythms are independent of menstrual cycles but contain
61 multidien structure in overlapping period bands, finite diary windows can still contain chance
62 alignments. Ratio definitions are especially vulnerable when seizure counts are sparse or
63 comparator phases contain few events. This creates a specificity question with direct
64 implications for clinical research: what apparent catamenial epilepsy rate should be expected
65 when no menstrual coupling exists?

66 We conducted a reproducible null-simulation study to estimate apparent catamenial epilepsy
67 classification across definitions and observation windows under independence. The analysis
68 evaluates specificity-like behavior only. Because traditional definitions combine C1, C2, and C3
69 patterns, we separately report C1/C2 and C3 false-positive behavior and compare negative-
70 binomial regression only against C1/C2 outcomes.

71 **Methods**

72 Study design. This Monte Carlo null simulation was defined before the full simulation run. For
73 each synthetic participant, seizure and menstrual/hormone diaries were generated independently
74 for 36 months. A participant-specific random circular shift of the seizure diary was sampled
75 uniformly over the diary length before merging to prevent hidden start-date alignment; cycle
76 labels were preserved from the hormone diary and seizure counts were wrapped with the shifted
77 seizure diary. Random calendar and cycle windows were sampled per participant and could
78 overlap across analysis families; all analyses were stratified by cohort and phase-labeling mode.

79 Cohorts. The healthy ovulatory cohort included 50,000 synthetic participants, age 18-45 years,
80 with ovulatory cycling enforced through the simulator adapter where available. The
81 heterogeneous menstruating-age cohort included 50,000 synthetic participants, age 13.0-54.9
82 years, allowing anovulation and irregularity. Because the hormone-cycle simulator exposed
83 medical-factor controls but not a natural prevalence sampler, PCOS, peri-menarche,
84 perimenopause, and dysmenorrhea were sampled from configuration rates. This cohort should be
85 interpreted as an assumption-driven heterogeneity stress test unless these rates are replaced with
86 externally validated prevalence inputs.

87 Diary simulators. CHOCOLATES (Cyclic Heterogeneous Overdispersed Clustered Open-source
88 L-relationship Adjustable Temporally limited E-diary Simulator) generated non-catamenial
89 seizure diaries using a hierarchical gamma-Poisson/negative-binomial seizure-count generator
90 with seizure-burden heterogeneity, multidien cycles, clustering, mean-variance structure, and
91 interseizure-interval limits.¹² HORMONE-CYCLE generated independent menstrual/hormone
92 diaries. It samples participant-level latent traits and cycle-level realizations, then interpolates
93 daily estradiol and progesterone trajectories from menstrual subphase reference medians. Its

94 features include cycle length, within-participant variability, bleeding days, ovulatory and
95 anovulatory cycles, follicular and luteal phase lengths, ovulation day, luteal progesterone,
96 inadequate-luteal-phase flags, and configured modifiers for PCOS, peri-menarche,
97 perimenopause, dysmenorrhea, oral contraceptive use, hormonal IUD, and copper IUD. Baseline
98 cycle targets were based on Apple Women's Health Study data, Natural Cycles data, and Stricker
99 et al. hormone reference values.¹³⁻¹⁵ Simulator validation targets, versions, and package
100 commits are documented in the Appendix following STRESS reporting principles.¹⁶

101 Phase labeling and definitions. Strict Herzog phases were primary. In a complete cycle of length
102 L, day 1 was menstrual-flow onset and backward day was $d-(L+1)$. Strict labels were assigned as
103 menstrual phase if forward day 1-3 or backward day -3 to -1; follicular phase if forward day 4-9;
104 ovulatory phase if forward day ≥ 10 and backward day ≤ -13 ; luteal phase if backward day -12
105 to -4; otherwise unlabeled. Because this strict ovulatory window expands with cycle length, we
106 added a sensitivity mode that fixed the ovulatory phase at backward days -16 to -13 immediately
107 before the luteal phase, with the follicular phase absorbing cycle-length variability.

108 Windowed Herzog thresholds used average daily seizure frequency (ADSF): $C1 =$
109 $\text{ADSF}(M)/\text{ADSF}(F+L) \geq 1.69$, $C2 = \text{ADSF}(O)/\text{ADSF}(F+L) \geq 1.83$, and $C3 =$
110 $\text{ADSF}(O+L+M)/\text{ADSF}(F) \geq 1.62$ among inadequate-luteal-phase days. C3 was evaluated only
111 in the heterogeneous cohort when `ilp_flag` was present; anovulatory or nonclassifiable cycles
112 without `ilp_flag` did not contribute to C3. We report any C1-C3 classification, the C1/C2 union,
113 mutually exclusive C1/C2/C3 categories, and C3-excluding endpoints.

114 Exact Herzog 2004 was restricted to complete 3-cycle windows with all cycles 23-35 days and
115 required at least two of three cycles to show any positive pattern. Other calendar and longer
116 cycle windows are reported as not evaluated for this rule. Herzog plus ≥ 4 seizure-day minimum

117 required at least 4 calendar months or 6 complete cycles plus at least 4 seizure days. Cycle
118 reproducibility required at least two-thirds of eligible cycles to be positive for the same pattern
119 and the corresponding pooled ratio to pass threshold.

120 Regression comparator. The negative-binomial (NB) comparator used a within-participant
121 generalized linear model with daily seizure count as outcome, log link, intercept, menstrual-
122 phase indicator, ovulatory-phase indicator, and cycle fixed effects when at least four complete
123 cycles were available. One-sided Wald tests evaluated C1 and C2 enrichment, Holm-adjusted
124 within participant across the two comparisons; positivity required adjusted $P < 0.05$ and the
125 corresponding Herzog rate-ratio threshold. C3 was not evaluated by NB regression. In this
126 revision, NB regression was evaluated for full-diary windows as a calibrated C1/C2 comparator.

127 The primary regression used full-diary method-of-moments dispersion as a stabilized
128 comparator, and a full-window, window-only dispersion sensitivity was added.

129 Statistical analysis. The primary outcome was the participant-window apparent catamenial
130 epilepsy rate among classifiable windows under the null. We also report positivity among all
131 attempted windows, participant-level positivity, indeterminate reasons, and rows with fewer than
132 1,000 classifiable windows as unstable and not interpreted. Calendar windows were expanded to
133 1, 3, 4, 6, 9, 12, 18, 24, and 36 months for practical monitoring-duration guidance. Study-level
134 Monte Carlo independently sampled 10,000 studies of 30, 50, and 100 participants without
135 replacement from each cohort, selecting one precomputed random valid 3-month window per
136 participant. Standard protocol approvals, registrations, and patient consents were not required
137 because all participants were synthetic.

Results

Simulation cohort and diary characteristics. The full run included 100,000 synthetic participants. With strict and luteal-anchored phase modes, expanded calendar windows, cycle windows, and study-pool windows, the analysis generated paired participant-window rows for sensitivity comparisons. Cohort summaries are shown in Table 1. The healthy ovulatory cohort had 100.0% ovulatory cycles by design. The heterogeneous cohort had 79.0% ovulatory cycles, greater cycle-length variability, and similar observed seizure burden.

C3 contribution and C1/C2 comparison. In strict-Herzog 36-month windows, the windowed Herzog threshold definition classified 11.2% of healthy ovulatory participants and 36.3% of heterogeneous-cohort participants as apparent catamenial epilepsy under independence (Table 2). In the heterogeneous cohort, the corresponding C1/C2 union was 11.8%, similar to the healthy cohort's 11.2%, whereas C3 positivity occurred in 37.0% of applicable heterogeneous-cohort windows. Thus the broad-cohort excess was primarily a C3/inadequate-luteal-phase phenomenon rather than a perimenstrual or periovulatory excess.

NB regression and denominator alignment. Stabilized-dispersion NB regression evaluated C1/C2 only and exhibited approximately nominal Type I error at 4.1% in the healthy ovulatory cohort and 4.2% in the heterogeneous cohort. The relevant comparison is therefore NB C1/C2 versus Herzog C1/C2, not NB versus the Herzog C1-C3 union. Calibration should not be interpreted as clinical usefulness; sensitivity and positive predictive value require true-coupling analyses outside this null study.

Observation-window effects. Short diary windows were most vulnerable to apparent catamenial classification. In random 3-month calendar windows, strict-Herzog windowed thresholds yielded apparent classification rates of 41.3% in the healthy ovulatory cohort and 51.2% in the

heterogeneous cohort. Practical calendar-window summaries through 36 months are shown in Figure 1. Exact Herzog 2004 applied to 3 complete cycles yielded apparent classification rates of 50.3% and 51.6% among classifiable windows, while many attempted 3-cycle windows were not classifiable. Rows with fewer than 1,000 classifiable windows were flagged as unstable and not interpreted.

Phase-labeling sensitivity and study-level Monte Carlo. The luteal-anchored ovulatory sensitivity tested whether strict Herzog's expanding ovulatory window drove periovulatory false positives; these results are reported in the notebook and Appendix. In 10,000 simulated 3-month studies, apparent prevalence varied with study size but could exceed historical benchmark values under the null. Figure 3 displays $n=30$, 50, and 100 distributions with the y-axis scaled as the proportion of simulated studies.

Discussion

This null simulation shows that apparent catamenial epilepsy can arise frequently even when seizure and menstrual processes are generated independently. The magnitude depends strongly on the catamenial pattern being tested, the window length, analyzability rules, and cohort menstrual-cycle assumptions. The largest heterogeneous-cohort excess was C3-driven; C1/C2 behavior was much more similar across cohorts. This distinction matters because C3 depends on inadequate-luteal-phase ascertainment rather than the perimenstrual or periovulatory windows that are usually easiest to audit from diaries alone.

The negative-binomial result should be interpreted as calibration, not superiority. A statistical test evaluated at $\alpha=0.05$ should produce approximately 5% positives under a correctly specified null, and the stabilized-dispersion regression did so. Ratio rules are not nominal-alpha tests and therefore have no comparable Type I error guarantee. The relevant comparison in this

184 paper is NB C1/C2 versus the Herzog C1/C2 union, because the NB model does not evaluate C3.
185 Sensitivity, positive predictive value, and clinical usefulness require true-coupling analyses and
186 are outside the scope of this null study.

187 These results help interpret the broad range of prevalence estimates in the catamenial epilepsy
188 literature without estimating true prevalence. Earlier work established biologically plausible and
189 clinically important menstrual-cycle seizure patterns, including C1, C2, and C3 patterns.¹⁻⁵
190 Treatment literature also suggests that robust perimenstrual exacerbation may identify subgroups
191 of interest, even though randomized evidence for hormonal therapy remains limited and
192 mixed.^{6,7} The present analysis instead quantifies a diagnostic background rate expected from
193 chance alignment when seizure cycles and menstrual cycles coexist under independence.

194 The findings are consistent with modern seizure-cycle research. Multidien seizure rhythms and
195 seizure forecasting studies have shown that seizure risk varies over days to weeks, and those
196 rhythms can be detected in long-term electronic diaries and chronic recordings.⁹⁻¹¹ A
197 menstrual-cycle analysis that does not account for intrinsic seizure cyclicity can therefore
198 mistake phase overlap for hormone coupling. This is most problematic when the observation
199 window is short, because a few events can dominate average daily seizure-frequency ratios.

200 Several methodologic implications follow. First, 3-month ratio-only classification should be
201 interpreted cautiously in research settings, particularly when seizure counts are sparse. Second,
202 C1, C2, and C3 should be reported separately rather than collapsed into a single prevalence
203 value. Third, studies should report both classifiable-window and all-attempted-window rates,
204 because stricter definitions may appear conservative partly by declining to classify insufficient
205 windows. Fourth, benchmark comparisons should match the original study design. For Herzog

2004, the design-matched comparison is the exact 3-complete-cycle rule, whereas random 3-calendar-month windows are a design-sensitivity analysis. Strict Herzog labeling was retained as the primary analysis for historical comparability. The luteal-anchored ovulatory sensitivity addresses a separate biologic question: whether the strict Herzog ovulatory window, which expands with longer cycles, creates excess periovulatory apparent classification. Presenting both phase modes separates historical reproducibility from a more anatomically fixed periovulatory definition. This study has limitations. The results are only as credible as the CHOCOLATES and HORMONE-CYCLE simulators, their parameter settings, and the adapter assumptions. The heterogeneous cohort medical-factor rates were configuration-sampled rather than drawn from a simulator-native prevalence sampler, and cycle-length variability is therefore a sensitivity driver rather than a settled population estimate. Monte Carlo binomial intervals do not include model-form uncertainty. This study does not evaluate false negatives, true prevalence, treatment effects, wavelet analyses, hidden Markov models, medication changes, or adjudicated seizure types. In conclusion, under an independence model with structured seizure diaries and menstrual-cycle diaries, apparent catamenial epilepsy classification can be common and can reach historical benchmark prevalence values. Future diagnostic and interventional studies should prespecify diary length, menstrual-phase labeling, analyzability thresholds, zero-denominator handling, ovulatory-status handling, and reproducibility criteria before interpreting apparent menstrual clustering as evidence of hormone-linked seizure exacerbation.

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232 Disclosure

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234 of Psychiatry and Neurology. SC, WK, and RS report no disclosures relevant to the manuscript.

235 Data Availability

236 The simulation code, configuration, analysis outputs, and machine-readable manifest are
237 available in the project repository. The reproducible run used master seed 20260505 and the
238 manifest in outputs/manifest.json records config parameters, output paths, file sizes, and
239 SHA-256 checksums. A versioned public archive with DOI will accompany the submitted
240 version.

241

References

1. Herzog AG, Harden CL, Liporace J, et al. Frequency of catamenial seizure exacerbation in women with localization-related epilepsy. *Ann Neurol*. 2004;56(3):431-434. doi:10.1002/ana.20214
2. Herzog AG, Klein P, Ransil BJ. Three patterns of catamenial epilepsy. *Epilepsia*. 1997;38(10):1082-1088. doi:10.1111/j.1528-1157.1997.tb01197.x
3. Herkes GK, Eadie MJ, Sharbrough F, Moyer T. Patterns of seizure occurrence in catamenial epilepsy. *Epilepsy Res*. 1993;15(1):47-52. doi:10.1016/0920-1211(93)90008-u
4. Herzog AG. Catamenial epilepsy: definition, prevalence pathophysiology and treatment. *Seizure*. 2008;17(2):151-159. doi:10.1016/j.seizure.2007.11.014
5. Herzog AG. Catamenial epilepsy: Update on prevalence, pathophysiology and treatment from the findings of the NIH Progesterone Treatment Trial. *Seizure*. 2015;28:18-25. doi:10.1016/j.seizure.2015.02.024
6. Maguire MJ, Nevitt SJ. Treatments for seizures in catamenial (menstrual-related) epilepsy. *Cochrane Database Syst Rev*. 2021;9(9):CD013225. doi:10.1002/14651858.CD013225.pub3
7. Herzog AG, Fowler KM, Smithson SD, et al. Progesterone vs placebo therapy for women with epilepsy: A randomized clinical trial. *Neurology*. 2012;78(24):1959-1966. doi:10.1212/WNL.0b013e318259e1f9
8. Reddy DS. The role of neurosteroids in the pathophysiology and treatment of catamenial epilepsy. *Epilepsy Res*. 2009;85(1):1-30. doi:10.1016/j.epilepsyres.2009.02.017
9. Baud MO, Kleen JK, Mirro EA, et al. Multi-day rhythms modulate seizure risk in epilepsy. *Nat Commun*. 2018;9(1):88. doi:10.1038/s41467-017-02577-y

- 264 10. Karoly PJ, Rao VR, Gregg NM, et al. Cycles in epilepsy. *Nat Rev Neurol*. 2021;17(5):267-
265 284. doi:10.1038/s41582-021-00464-1
- 266 11. Karoly PJ, Cook MJ, Maturana M, et al. Forecasting cycles of seizure likelihood. *Epilepsia*.
267 2020;61(4):776-786. doi:10.1111/epi.16485
- 268 12. Goldenholz DM, Westover MB. Flexible realistic simulation of seizure occurrence
269 recapitulating statistical properties of seizure diaries. *Epilepsia*. 2023;64(2):396-405.
270 doi:10.1111/epi.17471
- 271 13. Li H, Gibson EA, Jukic AMZ, et al. Menstrual cycle length variation by demographic
272 characteristics from the Apple Women's Health Study. *NPJ Digit Med*. 2023;6(1):100.
273 doi:10.1038/s41746-023-00848-1
- 274 14. Bull JR, Rowland SP, Scherwitsl EB, Scherwitsl R, Danielsson KG, Harper J. Real-world
275 menstrual cycle characteristics of more than 600,000 menstrual cycles. *NPJ Digit Med*.
276 2019;2:83. doi:10.1038/s41746-019-0152-7
- 277 15. Stricker R, Eberhart R, Chevailler MC, Quinn FA, Bischof P, Stricker R. Establishment of
278 detailed reference values for luteinizing hormone, follicle stimulating hormone, estradiol, and
279 progesterone during different phases of the menstrual cycle on the Abbott ARCHITECT
280 analyzer. *Clin Chem Lab Med*. 2006;44(7):883-887. doi:10.1515/CCLM.2006.160
- 281 16. Monks T, Currie CSM, Onggo BS, Robinson S, Kunc M, Taylor SJE. Strengthening the
282 reporting of empirical simulation studies: introducing the STRESS guidelines. *J Simul*.
283 2019;13(1):55-67. doi:10.1080/17477778.2018.1442155
- 284 17. Newmark ME, Penry JK. Catamenial epilepsy: a review. *Epilepsia*. 1980;21(3):281-300.
285 doi:10.1111/j.1528-1157.1980.tb04074.x

- 286 18. Duncan S, Read CL, Brodie MJ. How common is catamenial epilepsy? *Epilepsia*.
287 1993;34(5):827-831. doi:10.1111/j.1528-1157.1993.tb02097.x

288

289 **Tables**

290 Table 1. Simulated cohort characteristics.

Cohort	N	Mean age, y	Mean cycle length, d	Cycle-length SD, d	Ovulatory cycles	Seizure days/month	Seizures/month
healthy ovulatory	50,000	31.5 (SD 7.8)	29.15	3.39	100.0%	2.46	6.83
heterogeneous menstruating-age	50,000	34.0 (SD 12.1)	30.92	4.74	79.0%	2.45	6.80

291

292 Values are cohort-level means from the full 100,000-participant run. Cycle-length SD is the

293 mean within-participant cycle-length SD.

294 Table 2. Participant-window false-positive results.

Cohort	Analysis setting	Definition	Classifiable / attempted	Positive among classifiable	Positive among all attempted	Indeterminate
healthy ovulatory	36-month full diary	Windowed Herzog thresholds	49,605 / 50,000	11.2% (11.0%-11.5%)	11.2%	0.8%
healthy ovulatory	36-month full diary	Windowed Herzog C1/C2 union	49,605 / 50,000	11.2% (11.0%-11.5%)	11.2%	0.8%
healthy ovulatory	36-month full diary	Windowed Herzog C3 only	0 / 50,000	NA	0.0%	100.0%
healthy ovulatory	36-month full diary	Herzog C1/C2 plus ≥ 4 seizure-day minimum	48,359 / 50,000	10.2% (9.9%-10.5%)	9.9%	3.3%
healthy ovulatory	36-month full diary	Negative-binomial regression C1/C2	48,359 / 50,000	4.1% (3.9%-4.2%)	3.9%	3.3%
healthy ovulatory	3-month calendar window	Windowed Herzog thresholds	45,280 / 50,000	41.3% (40.8%-41.7%)	37.4%	9.4%
healthy ovulatory	3 complete cycles	Exact Herzog 2004, any pattern	22,996 / 50,000	49.7% (49.0%-50.3%)	22.8%	54.0%
heterogeneous menstruating-age	36-month full diary	Windowed Herzog thresholds	49,587 / 50,000	36.3% (35.8%-36.7%)	36.0%	0.8%

Cohort	Analysis setting	Definition	Classifiable / attempted	Positive among classifiable	Positive among all attempted	Indeterminate
heterogeneous menstruating-age	36-month full diary	Windowed Herzog C1/C2 union	49,587 / 50,000	11.8% (11.5%-12.1%)	11.7%	0.8%
heterogeneous menstruating-age	36-month full diary	Windowed Herzog C3 only	38,989 / 50,000	37.0% (36.5%-37.5%)	28.8%	22.0%
heterogeneous menstruating-age	36-month full diary	Herzog C1/C2 plus ≥ 4 seizure-day minimum	48,333 / 50,000	10.7% (10.5%-11.0%)	10.4%	3.3%
heterogeneous menstruating-age	36-month full diary	Negative-binomial regression C1/C2	48,333 / 50,000	4.2% (4.0%-4.4%)	4.1%	3.3%
heterogeneous menstruating-age	3-month calendar window	Windowed Herzog thresholds	45,297 / 50,000	51.2% (50.7%-51.6%)	46.4%	9.4%
heterogeneous menstruating-age	3 complete cycles	Exact Herzog 2004, any pattern	16,757 / 50,000	51.2% (50.4%-51.9%)	17.2%	66.5%

295

296 All rows use strict Herzog phase labeling. Rows with fewer than 1,000 classifiable windows are

297 flagged in the analysis outputs and not interpreted.

298 Table 3. C1/C2/C3 mutually exclusive pattern decomposition.

Cohort	Definition	C1 only	C2 only	C1+C2	C3 only	C3 plus C1/C2	None	Indeterminate
healthy ovulatory	Windowed Herzog thresholds	5.6%	3.4%	2.1%	0.0%	0.0%	88.1%	0.8%
healthy ovulatory	Herzog plus ≥ 4 seizure-day minimum	5.1%	2.9%	1.9%	0.0%	0.0%	86.8%	3.3%
heterogeneous menstruating-age	Windowed Herzog thresholds	3.8%	2.1%	1.2%	24.3%	4.6%	63.2%	0.8%
heterogeneous menstruating-age	Herzog plus ≥ 4 seizure-day minimum	3.5%	1.7%	1.0%	24.0%	4.2%	62.3%	3.3%

299

Percentages are among all attempted 36-month full-diary windows, so indeterminate windows remain visible in the denominator.

Table 4. Study-level Monte Carlo summaries for 3-month windows.

Cohort	N per study	Definition	Mean apparent prevalence	2.5th-97.5th percentiles	P >=39.1%	P >=44.2%
healthy ovulatory	30	Windowed Herzog thresholds	37.5%	20.0%-53.3%	45.4%	20.1%
healthy ovulatory	30	Windowed Herzog C1/C2 union	37.5%	20.0%-53.3%	45.4%	20.1%
healthy ovulatory	50	Windowed Herzog thresholds	37.3%	24.0%-50.0%	39.7%	13.0%
healthy ovulatory	50	Windowed Herzog C1/C2 union	37.3%	24.0%-50.0%	39.7%	13.0%
healthy ovulatory	100	Windowed Herzog thresholds	37.5%	28.0%-47.0%	33.8%	7.1%
healthy ovulatory	100	Windowed Herzog C1/C2 union	37.5%	28.0%-47.0%	33.8%	7.1%
heterogeneous menstruating-age	30	Windowed Herzog thresholds	46.3%	30.0%-63.3%	80.5%	55.9%
heterogeneous menstruating-age	30	Windowed Herzog C1/C2 union	38.2%	20.0%-56.7%	48.9%	22.4%
heterogeneous menstruating-age	50	Windowed Herzog thresholds	46.1%	32.0%-60.0%	84.5%	55.9%
heterogeneous menstruating-age	50	Windowed Herzog C1/C2 union	38.1%	26.0%-52.0%	43.9%	15.3%
heterogeneous menstruating-age	100	Windowed Herzog thresholds	46.0%	36.0%-56.0%	90.8%	62.2%
heterogeneous menstruating-age	100	Windowed Herzog C1/C2 union	38.0%	29.0%-47.0%	38.2%	8.8%

304 Study-level rows summarize 10,000 simulated studies per cohort and sample size using one
305 random valid 3-month window per selected participant. N=30 is illustrative; N=50 and N=100
306 are sensitivity rows.

307

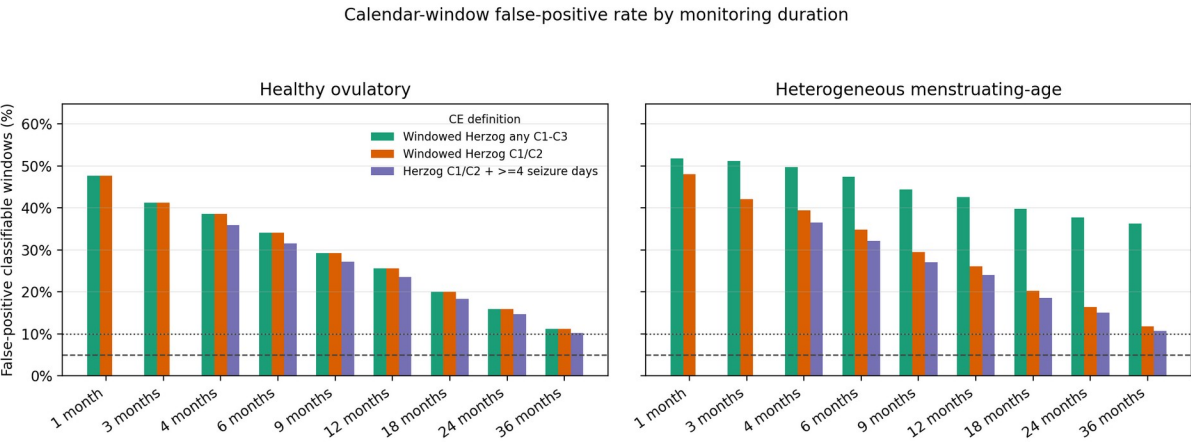
308 **Figure Legends**

309 Figure 1. False-positive rates by observation window.

310 Bars show the percentage of classifiable calendar-window participant windows classified as

311 catamenial epilepsy under the null using strict Herzog phase labeling. Dashed and dotted

312 horizontal lines mark 5% and 10% reference rates.



313

314 Figure 2. C1/C2/C3 mutually exclusive pattern decomposition.

315 Stacked bars show mutually exclusive pattern categories for 36-month full-diary windows by

316 cohort and definition. Percentages use all attempted participant windows as the denominator.

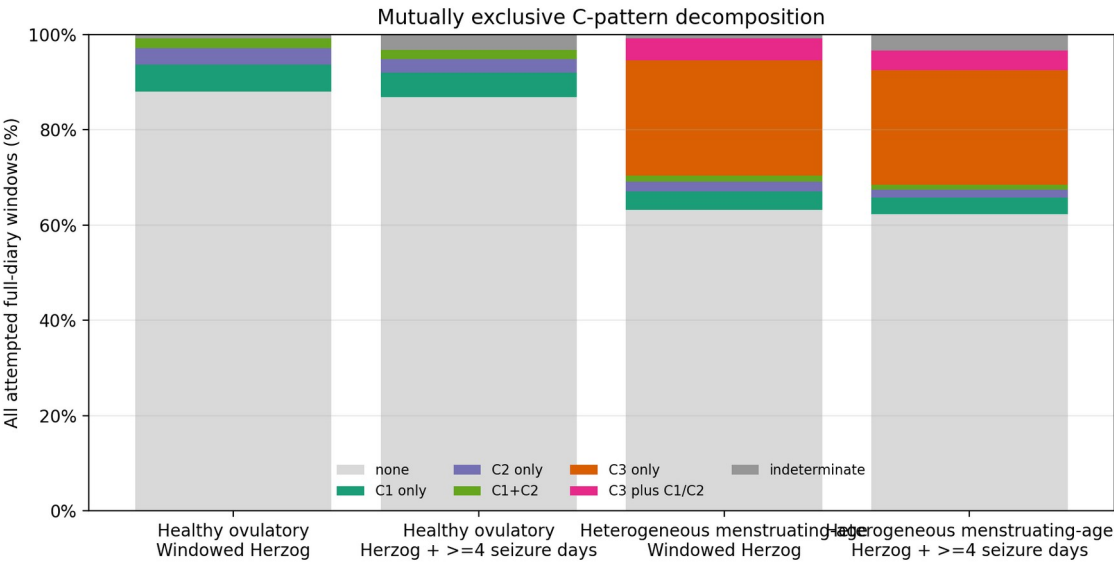


Figure 3. Apparent prevalence in null studies. Distributions show apparent catamenial epilepsy prevalence in 10,000 simulated 3-month studies using strict Herzog windowed thresholds. Curves are shown for n=30, n=50, and n=100 participants; vertical lines show benchmark prevalence values of 39.1% and 44.2%.

